

Mesh slings for bladder leakage and mesh for prolapse help many people, and most never have a problem with them. A small number later have pain, mesh showing through the vaginal wall, trouble emptying or other problems. These can be checked and treated. Mesh that is not causing problems **does not need to be removed**.

About Pelvic Mesh

- **Midurethral sling:** a narrow mesh tape placed under the urethra to stop leaks with coughing or exercise. Slings are still widely used.
- **Vaginal mesh for prolapse:** placed through the vagina. Its sale stopped in the United States in 2019. This does not mean mesh already in place must come out.
- **Sacrocolpopexy mesh:** placed through the belly to lift the top of the vagina. It is still used.

Benefits and Risks in Numbers

about

2 in 100

had sling mesh show through the vaginal wall (about 4 in 100 with prolapse mesh)

3 in 4

felt better after mesh removal for exposure alone, vs 1 in 3 for pain alone

1 in 5

had leakage return after part of a sling was removed, vs about 1 in 2 after all of it

Numbers are averages from published studies. Your own risk may differ; ask your team.

Signs of a Mesh Problem

- Vaginal bleeding, spotting or discharge that keeps coming back
- Feeling a rough edge in the vagina, or a partner feeling scratching or pain during sex
- Pain in the pelvis, groin or thigh, or pain with sex
- A weak stream, trouble emptying, new urgency, or repeated bladder infections
- Blood in the urine, or bleeding or discharge from the anus. After sacrocolpopexy, new or lasting low back pain

Most problems show up in the first year, but some appear years later.

Learn the Terms

Mesh

A permanent woven material, usually polypropylene, used to support tissue.

Exposure

Mesh that can be seen or felt through the vaginal wall. Also called erosion.

Extrusion

Mesh that pokes out into the vagina.

Perforation

Mesh that has worked into the bladder, urethra or bowel.

Sling release

Cutting or loosening a sling that blocks the flow of urine.

Partial or complete removal

Taking out part of the mesh, or as much of it as can safely be removed.

Urogynecologist

A specialist in pelvic floor problems. Some have special experience with mesh problems.

Will removing the mesh fix it? It depends on the problem. Mesh exposure usually gets better with treatment. Pain without exposure is harder to predict, and removal may not relieve it. Leakage can come back after a sling is cut or removed. In one study of people treated at specialist centers, 6 in 10 needed two or more treatments.

How Problems Are Checked

- Your history and a pelvic exam to find tender spots or exposed mesh. **Bring your surgery records**, including the name of the mesh product if you have it.
- A camera look into the bladder (cystoscopy) if you have bladder symptoms or blood in the urine.
- Sometimes an ultrasound or MRI to see where the mesh lies.

Treatment Options

Watching

A small exposure that causes no symptoms may simply be watched.

Vaginal estrogen cream

A 6–12-week trial may help a very small exposure heal.

Trimming or covering

The exposed piece is cut away and the vaginal skin closed, sometimes in the office.

Sling release

For trouble emptying after a sling. It may be done within weeks of surgery or later.

Partial or complete removal

Through the vagina, the belly, the groin or thigh, or with a camera. Prolapse or leakage can return and may need another repair.

Pain care

Pelvic floor physical therapy and pain specialists can help, with or without surgery.

Call your care team if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Vaginal bleeding, discharge or pain that is new or getting worse
- Trouble emptying your bladder, or repeated bladder infections

Go to an emergency department if:

- You cannot urinate and your bladder feels full or painful
- Fever with severe pelvic, belly or back pain
- Heavy bleeding
- You cannot reach the office and are worried

Three Things to Remember

1. Most people with mesh have no problems. Mesh that causes no symptoms does not need removal.
2. Exposure usually improves with treatment. Pain alone is harder to fix.
3. See a specialist experienced with mesh, and bring your surgery records.

Questions to Ask

- What kind of mesh do I have, and where?
- What is the goal of treatment for me?
- How much mesh would come out?
- Have you treated problems like mine?